

Locum Tenens Vetting Guide

How to verify credentials, assess agencies, and avoid unqualified temporary physicians

- ✓ Credentialing requirements and verification timelines explained
- ✓ Board certification, DEA, and state license verification process
- ✓ 10 red flags that indicate a problematic locum agency or provider
- ✓ Sample vetting questionnaire you can send to agencies

Understanding Locum Tenens Credentialing

Credentialing a locum tenens provider involves verifying identity, education, training, licensure, board certification, malpractice history, and references. The process typically takes 30–90 days for a new provider at a new facility. Understanding each component helps you set realistic timelines and catch gaps before the provider starts seeing patients.

Credentialing Requirements

Category	Documents Required	Verification Source
Identity	Government-issued photo ID, SSN, NPI	NPES (NPI Registry)
Education	Medical school diploma, residency/fellowship certificates	AMA Physician Masterfile, medical school directly
Licensure	Active state medical license(s) for practicing state	State medical board website
Board Certification	ABMS or AOA board certification	ABMS Certification Matters (certificationmatters.org)
DEA Registration	Active DEA certificate for controlled substances	DEA CSOS or agency verification
Malpractice History	10-year malpractice claims history	NPDB (National Practitioner Data Bank) query
References	2-3 peer references from recent clinical work	Direct contact by credentialing team
Work History	CV covering 5-10 years with no gaps	Direct verification with prior employers
Background Check	Criminal history, OIG exclusion, SAM exclusion	OIG LEIE, SAM.gov, state criminal databases
Health Screening	TB test, immunizations, drug screen, COVID vaccination	Facility-specific requirements

Board Certification Explained

Board Certification: What It Means and How to Verify

Board certification through the American Board of Medical Specialties (ABMS) or the American Osteopathic Association (AOA) demonstrates that a physician has completed residency training and passed rigorous examinations in their specialty.

ABMS Member Boards

Board	Specialties Covered	Initial Cert Exam
ABIM (Internal Medicine)	IM, Cardiology, Pulm/Crit Care, GI, Endocrine, Oncology	Written exam after residency
ABEM (Emergency Medicine)	Emergency Medicine	Written + oral exam
ABFM (Family Medicine)	Family Medicine, Sports Medicine	Written exam
ABS (Surgery)	General Surgery, Vascular, Surgical Critical Care	Written + oral exam
ABP (Pediatrics)	Pediatrics, Neonatal-Perinatal, Peds Cardiology	Written exam
ABA (Anesthesiology)	Anesthesiology, Pain Medicine, Critical Care	Written + oral exam
ABPN (Psychiatry & Neurology)	Psychiatry, Neurology, Child Psychiatry	Written exam
ABR (Radiology)	Diagnostic Radiology, Radiation Oncology	Core + certifying exams
ABOG (OB/GYN)	Obstetrics & Gynecology	Written + oral exam
ABPath (Pathology)	Anatomic Pathology, Clinical Pathology	Written exam

Maintenance of Certification

Element	Requirement
License & Professional Standing	Active, unrestricted state medical license
Lifelong Learning & Self-Assessment	Ongoing CME credits (specialty-specific requirements)

Element	Requirement
Knowledge Assessment	Periodic secure exam or continuous assessment
Practice Performance	Quality improvement activities
Cycle length	Typically 10 years (some boards moving to continuous)

"Board Eligible" vs. "Board Certified"

A provider who is "board eligible" has completed residency but has NOT passed the board exam. For locum assignments, most facilities require board certification — not just eligibility. If a provider has been board eligible for 5+ years without certifying, this warrants scrutiny.

Verification Process — Step by Step

Step-by-step verification

Step 1: NPI Verification. Search the NPPES NPI Registry (npiregistry.cms.hhs.gov) to confirm the provider's NPI number, name, specialty, and practice address.

Step 2: State Medical License. Verify through the practicing state's medical board website. Confirm the license is active, unrestricted, and has no disciplinary actions. Check ALL states where the provider holds or has held a license.

Step 3: Board Certification. Verify through ABMS Certification Matters (certificationmatters.org) — free public lookup. Confirm specialty, certification status (active vs. expired), and MOC participation.

Step 4: NPDB Query. The National Practitioner Data Bank (NPDB) contains malpractice payment reports, adverse licensure actions, and clinical privilege restrictions. Facilities can query directly; agencies should provide the query report.

Step 5: OIG/SAM Exclusion Check. Search the OIG LEIE (exclusions.oig.hhs.gov) and SAM.gov to confirm the provider is not excluded from federal healthcare programs. Employing an excluded provider can result in penalties up to \$100,000 per item/service.

Step 6: DEA Verification. Confirm active DEA registration for the state(s) where the provider will practice. DEA registrations are state-specific.

Step 7: Reference Checks. Contact 2-3 peer references directly. Ask about clinical competency, communication, reliability, and whether they would work with this provider again.

Step 8: Malpractice History Review. Review the provider's 10-year malpractice claims history. A single settled claim may not be disqualifying, but patterns of claims, especially in the same clinical area, are red flags.

OIG Exclusion Penalty

Billing Medicare/Medicaid for services provided by an excluded individual can result in Civil Monetary Penalties of up to \$100,000 per item or service, plus treble damages and program exclusion for your facility. Check OIG LEIE before EVERY new provider starts.

Credentialing Timeline

Realistic Credentialing Timelines by Scenario

Scenario	Typical Timeline	Key Bottleneck
New provider, has state license, standard credentialing	30–45 days	Hospital committee meeting schedule
New provider, needs new state license	60–120 days	State medical board processing time
New provider, California license needed	90–150 days	California processing backlog
Disaster/emergency privileges	24–72 hours	Limited to emergency declarations
Telehealth-only (IMLC compact state)	14–30 days	If provider holds IMLC license
Re-credentialing (returning locum, same facility)	7–14 days	Update verification, no full re-credential

Interstate Medical Licensure Compact

IMLC Compact

As of 2026, 42 states participate in the Interstate Medical Licensure Compact, which provides an expedited pathway for physicians to obtain licenses in multiple states. IMLC can reduce multi-state licensing from months to weeks. Check imlcc.org for participating states.

10 Red Flags of a Problematic Locum Agency or Provider

1

Agency won't disclose the clinician's credentials before you sign.

You should be able to review the provider's CV, board certification status, and license before committing.

2

Gaps in work history with no explanation.

Unexplained gaps of 6+ months in a physician's CV may indicate suspension, health issues, or disciplinary action. Always ask.

3

"Board eligible" for 5+ years.

If a provider completed residency years ago and hasn't passed boards, ask why. Most boards have time limits on eligibility.

4

Multiple state license restrictions or disciplinary actions.

One resolved complaint may be normal. Patterns across multiple states are a red flag. Check every state.

5

Agency provides claims-made (not occurrence) malpractice coverage.

Claims-made policies leave your facility exposed after the locum departs unless tail coverage is purchased (at \$5,000–\$25,000+). Occurrence policies cover regardless of when the claim is filed.

6

No NPDB query provided.

The agency should run an NPDB query for every provider and share the results. Refusal to share NPDB results is a serious red flag.

7

Agency pressures you to skip credentialing steps.

"We've already verified everything" is not sufficient. Your facility has an independent obligation to verify credentials per CMS Conditions of Participation.

8

Provider has never used your EMR system.

A locum unfamiliar with your EMR (Epic, Cerner, MEDITECH, etc.) will be significantly slower for the first 1-2 weeks. Ask about EMR experience upfront.

9

Excessively high markup with no transparency.

If the agency won't provide any breakdown of the bill rate, and the rate is significantly above market for the specialty, investigate alternatives.

10

No 24/7 agency support line.

A legitimate locum agency provides round-the-clock support for clinical and logistical issues during the assignment. No after-hours support means problems at 2 AM are your problem.

Agency vs. Direct Hire Comparison

Two-column layout

USE A LOCUM TENENS AGENCY WHEN

- Urgent coverage need (< 30 days)
- Short-term gap coverage (vacation, leave, recruitment)
- Trying out a provider before permanent hire
- Hard-to-fill specialty or rural location
- Need credentialing and compliance handled externally
- Seasonal volume fluctuations
- New facility or service line launch

CONSIDER DIRECT CONTRACTING WHEN

- Long-term need (6+ months) with identified provider
- Provider already credentialed at your facility
- Budget-constrained and willing to handle logistics in-house
- Strong internal credentialing and compliance infrastructure

Cost comparison

Factor	Agency Locum	Direct Contract
Bill rate	\$225/hr	N/A
Clinician pay	~\$150/hr (est.)	\$175/hr (direct pay, no agency cut)
Total cost (40 hrs/wk × 13 wks)	\$117,000	\$91,000
Housing (agency-arranged vs. self)	Included	\$7,500 (3 months housing)
Travel	Included	\$3,000 (est.)
Malpractice	Included	\$2,500–\$5,000
Credentialing	Agency handles	Your staff handles (40-80 hours)
Total all-in cost	\$117,000	\$104,000

Factor	Agency Locum	Direct Contract
Administrative burden	Low	High

Sample Vetting Questionnaire

Locum Tenens Agency Vetting Questionnaire — Send This Before Contracting

Copy and send these questions to any locum tenens agency you're considering. Their answers will tell you everything you need to know about their quality and transparency.

1. Provider Credentials

- Will you provide the provider's full CV, board certification verification, and license status before we sign a contract?
- Do you run an NPDB query on every provider? Will you share the results with us?
- Do you verify the provider's work history for the past 10 years with no gaps?
- What is your process for checking OIG/SAM exclusion lists?

2. Malpractice & Insurance

- Do you provide occurrence-based or claims-made malpractice coverage?
- If claims-made, who pays for tail coverage when the assignment ends?
- What are the policy limits? (Standard: \$1M per occurrence / \$3M aggregate)
- Will you provide a Certificate of Insurance naming our facility?

3. Credentialing & Compliance

- What is your average credentialing turnaround time for our state?
- Do you handle state medical license procurement if the provider doesn't hold one?
- How do you verify DEA registration, board certification, and immunization status?
- Do you perform criminal background checks and drug screens on all providers?

4. Pricing & Transparency

- What is the all-in bill rate, and what does it include (housing, travel, malpractice)?
- Will you disclose the agency markup percentage or the clinician's pay rate?
- What are the overtime, call coverage, and holiday rates?
- What is your cancellation policy (both sides) and conversion fee?

5. Operations & Support

- Do you provide 24/7 support during the assignment?

- What is your process if the provider is not a good clinical or cultural fit?
- How quickly can you provide a replacement if the provider leaves mid-assignment?
- What EMR systems has the proposed provider used? Will you provide EMR training?

6. References

- Can you provide 3 facility references for similar specialty placements?
- What is your provider retention rate (% who complete full assignments)?

LocumTrust

LocumTrust

The most comprehensive directory of locum tenens providers in the United States

Find agencies at locumtrust.com